Pearson Edexcel

Mark Scheme (Results)

Summer 2022

Pearson Edexcel GCSE
In History (1HIA)

Paper 1: Thematic study and historic environment (1HIA/11)

Option 11: Medicine in Britain, c1250–present and
The British sector of the Western Front, 1914–18: injuries, treatment and the trenches

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General Marking Guidance

- All candidates must receive the same treatment. Examiners must mark the first candidate in exactly the same way as they mark the last.
- Mark schemes should be applied positively. Candidates must be rewarded for what they have shown they can do rather than penalised for omissions.
- Examiners should mark according to the mark scheme not according to their perception of where the grade boundaries may lie.
- There is no ceiling on achievement. All marks on the mark scheme should be used appropriately.
- All the marks on the mark scheme are designed to be awarded. Examiners should always award full marks if deserved, i.e. if the answer matches the mark scheme. Examiners should also be prepared to award zero marks if the candidate's response is not worthy of credit according to the mark scheme.
- Where some judgement is required, mark schemes will provide the principles by which marks will be awarded and exemplification may be limited.
- When examiners are in doubt regarding the application of the mark scheme to a candidate's response, the team leader must be consulted.
- Crossed out work should be marked UNLESS the candidate has replaced it with an alternative response.

How to award marks when level descriptions are used

1. Finding the right level

The first stage is to decide which level the answer should be placed in. To do this, use a 'best-fit' approach, deciding which level most closely describes the quality of the answer. Answers can display characteristics from more than one level, and where this happens markers must use the guidance below and their professional judgement to decide which level is most appropriate.

For example, one stronger passage at L4 would not by itself merit a L4 mark, but it might be evidence to support a high L3 mark, unless there are substantial weaknesses in other areas. Similarly, an answer that fits best in L3 but which has some characteristics of L2 might be placed at the bottom of L3. An answer displaying some characteristics of L3 and some of L1 might be placed in L2.

2. Finding a mark within a level

After a level has been decided on, the next stage is to decide on the mark within the level. The instructions below tell you how to reward responses within a level. However, where a level has specific guidance about how to place an answer within a level, always follow that guidance.

Levels containing two marks only

Start with the presumption that the work will be at the top of the level. Move down to the lower mark if the work only just meets the requirements of the level.

Levels containing three or more marks

Markers should be prepared to use the full range of marks available in a level and not restrict marks to the middle. Markers should start at the middle of the level (or the upper-middle mark if there is an even number of marks) and then move the mark up or down to find the best mark. To do this, they should take into account how far the answer meets the requirements of the level:

- If it meets the requirements *fully*, markers should be prepared to award full marks within the level. The top mark in the level is used for answers that are as good as can realistically be expected within that level
- If it only *barely* meets the requirements of the level, markers should consider awarding marks at the bottom of the level. The bottom mark in the level is used for answers that are the weakest that can be expected within that level
- The middle marks of the level are used for answers that have a *reasonable* match to the descriptor. This might represent a balance between some characteristics of the level that are fully met and others that are only barely met.

Indicative content

Examiners are reminded that indicative content is provided as an illustration to markers of some of the material that may be offered by students. It does not show required content and alternatives should be credited where valid.

The British sector of the Western Front, 1914–18: injuries, treatment and the trenches

Question	
1	Describe **two** features of the underground hospital at Arras. **Target**: knowledge of key features and characteristics of the period. **AO1**: 4 marks.
Marking instructions	
Award 1 mark for each valid feature identified up to a maximum of two features. The second mark should be awarded for supporting information. e.g. • *Existing caves and tunnels were extended **(1).** A hospital was created in preparation for casualties expected after an attack on the German lines in 1917 **(1).*** • *The hospital complex was well equipped **(1).** It included water, electricity, capacity for 700 patients, operating theatres, and a mortuary **(1).*** • *It was close to the front line of the attack at Arras in 1917 **(1).** Treatment for the wounded could be quickly accessed and would give the injured a better chance of survival **(1)**.* Accept other appropriate features and supporting information.	

Question		
2 (a)		How useful are Sources A and B for an enquiry into the problem of trench foot? Explain your answer, using Sources A and B and your knowledge of the historical context. **Target**: Analysis and evaluation of source utility. **AO3**: 8 marks.
Level	**Mark**	**Descriptor**
	0	No rewardable material.
1	**1–2**	• A simple judgement on utility is given, and supported by undeveloped comment on the content of the sources and/or their provenance[1]. Simple comprehension of the source material is shown by the extraction or paraphrase of some content. Limited contextual knowledge is deployed with links to the sources.
2	**3–5**	• Judgements on source utility for the specified enquiry are given, using valid criteria. Judgements are supported by developed comment related to the content of the sources and/or their provenance[1]. Comprehension and some analysis of the sources is shown by the selection and use of material to support comments on their utility. Contextual knowledge is used directly to support comments on the usefulness of the content of the sources and/or their provenance.
3	**6–8**	• Judgements on source utility for the specified enquiry are given, applying valid criteria with developed reasoning which takes into account how the provenance[1] affects the usefulness of the source content. The sources are analysed to support reasoning about their utility. Contextual knowledge is used in the process of interpreting the sources and applying criteria for judgements on their utility.

Notes
1. Provenance = nature, origin, purpose.

Marking instructions

Markers must apply the descriptors above in line with the general marking guidance (page 3).

No credit may be given for contextual knowledge unless it is linked to evaluation of the sources.

No credit may be given for generic comments on provenance which are not used to evaluate source content.

Indicative content guidance

Answers must be credited according to candidates' deployment of material in relation to the qualities outlined in the mark scheme. While specific references are made in the indicative content below, this does not imply that these must be included; other relevant material must also be credited. The grouping of points below does not imply that this is how candidates are expected to structure their answers.

Source A

The usefulness could be identified in terms of the following points which could be drawn from the source:

- Source A is useful because it describes the conditions in the trenches that led to soldiers developing trench foot.
- Source A is useful because it demonstrates the attempts of the army to prevent soldiers developing trench foot.
- It is useful in the way it suggests that trench foot was a serious problem affecting the health of large numbers of the soldiers.

The following points could be made about the authorship, nature or purpose of the source and applied to ascribe usefulness to material drawn from it:

- Captain Impey was an officer, so he would have been responsible for the health of soldiers under his command and he would have had a reliable overview of their health problems.
- The source was written after the war, which suggests that he remembers trench foot as a significant problem.

Knowledge of the historical context should be deployed to support inferences and/or to assess the usefulness of information. Relevant points may include:

- Soldiers would be on the front-line trench in muddy conditions for several days with no chance to get clean and dry until they moved back to the support or reserve trenches.
- Whale oil was recommended for use as a way of preventing trench foot.
- **Source B**

The usefulness could be identified in terms of the following points which could be drawn from the source:

- Source B is useful because it suggests that the army took trench foot seriously, as a formal foot inspection was being carried out by the Medical Officer.
- The content of Source B is useful as it shows a cause of trench foot, the base of the support trench was earth, which would become muddy in wet weather.

The following points could be made about the authorship, nature or purpose of the source and applied to ascribe usefulness to material drawn from it:

- The photograph was taken in 1918, so it can be used to show that the army had developed measures to prevent trench foot developing.
- It is possible that the photograph was used to reassure people at home or army senior ranks that action was being taken to deal with the problem of trench foot.

Knowledge of the historical context should be deployed to support inferences and/or to assess the usefulness of information. Relevant points may include:

- The army took trench foot seriously because it was a form of gangrene and, in severe cases, resulted in amputation.
- Some sections of the trenches were in low-lying ground, which did not drain well, for example the trenches at Passchendaele were waterlogged and muddy.

Question	
2 (b)	How could you follow up Source A to find out more about the problem of trench foot? In your answer, you must give the question you would ask and the type of source you could use. **Target**: Source analysis and use (the ability to frame historical questions). **AO3**: 4 marks.

Marking instructions

Award 1 mark for selecting a detail in Source A that could form the basis of a follow-up enquiry and 1 mark for an appropriate follow-up question.

e.g.

Detail in Source A that I would follow up*: The provision of dry socks and rubber boots.* ***(1)***

- Question I would ask*: Was this effective in preventing trench foot?* ***(1)***

(No mark for a question that is **not** linked to following up Source A, e.g. '*because it would be an interesting question to ask*'.)

Award 1 mark for identification of an appropriate source to use in a follow-up enquiry and 1 mark for an answer that explains how the information it contains could help answer the chosen follow-up question.

e.g.

- What type of source I would look for*: British Army War diaries.* ***(1)***
- How this might help answer my question*: Extracts from a range of diaries could give us individual soldiers' description of the condition of their feet, providing an overview of what measures were taken and how effective they were.* ***(1)***

Accept other appropriate alternatives.

Medicine in Britain, c1250–present

Question		
3		Explain **one** way in which attempts to prevent the spread of infectious diseases in the period c1500-c1700 were similar to attempts to prevent the spread of infectious diseases in the period c1700—c1900. **Target**: Analysis of second order concepts: similarity [AO2]; Knowledge and understanding of features and characteristics of the period [AO1]. **AO2:** 2 marks. **AO1:** 2 marks.
Level	**Mark**	**Descriptor**
	0	No rewardable material.
1	**1–2**	• Simple or generalised comment is offered about a similarity. [AO2] • Generalised information about the topic is included, showing limited knowledge and understanding of the periods. [AO1]
2	**3–4**	• Features of the period are analysed to explain a similarity. [AO2] • Specific information about the topic is added to support the comparison, showing good knowledge and understanding of the periods. [AO1]

Marking instructions

Markers must apply the descriptors above in line with the general marking guidance (page 3).

Performance in AO1 and AO2 is interdependent. An answer displaying **no** qualities of AO2 cannot be awarded more than the top of Level 1, no matter how strong performance is in AO1; markers should note that the expectation for AO1 is that candidates demonstrate both knowledge *and* understanding.

Indicative content guidance

Answers must be credited according to candidates' deployment of material in relation to the qualities outlined in the mark scheme. While specific references are made in the indicative content below, this does not imply that these must be included; other relevant material must also be credited.

Relevant points may include:

- In both cases, attempts were made to prevent the spread of disease by purifying the air. In 1665, the Lord Mayor of London ordered rubbish to be removed and in the eighteenth and nineteenth centuries, barrels of tar were burned.
- In both cases, the authorities passed laws to try to prevent the spread of the disease. In 1665, the Lord Mayor of London ordered theatres to be closed to prevent people gathering. In the eighteenth and nineteenth centuries, the government passed laws to enforce the use of Jenner's vaccination.

Question		
4		Explain why attitudes towards surgery changed in the period c1800-present. You **may** use the following in your answer: • anaesthetics • high-tech surgery You **must** also use information of your own. **Target**: Analysis of second order concepts: causation/change [AO2]; Knowledge and understanding of features and characteristics [AO1]. **AO2**: 6 marks. **AO1**: 6 marks.
Level	**Mark**	**Descriptor**
	0	No rewardable material.
1	**1–3**	• A simple or generalised answer is given, lacking development and organisation. [AO2] • Limited knowledge and understanding of the topic is shown. [AO1]
2	**4–6**	• An explanation is given, showing limited analysis and with implicit or unsustained links to the conceptual focus of the question. It shows some development and organisation of material, but a line of reasoning is not sustained. [AO2] • Accurate and relevant information is included, showing some knowledge and understanding of the period. [AO1] *Maximum 5 marks for Level 2 answers that do not go beyond aspects prompted by the stimulus points*
3	**7–9**	• An explanation is given, showing some analysis, which is mainly directed at the conceptual focus of the question. It shows a line of reasoning that is generally sustained, although some passages may lack coherence and organisation. [AO2] • Accurate and relevant information is included, showing good knowledge and understanding of the required features or characteristics of the period studied. [AO1] *Maximum 8 marks for Level 3 answers that do not go beyond aspects prompted by the stimulus points.*
4	**10–12**	• An analytical explanation is given which is directed consistently at the conceptual focus of the question, showing a line of reasoning that is coherent, sustained and logically structured. [AO2] • Accurate and relevant information is precisely selected to address the question directly, showing wide-ranging knowledge and understanding of the required features or characteristics of the period studied. [AO1] *No access to Level 4 for answers which do not go beyond aspects prompted by the stimulus points.*

Marking instructions

Markers must apply the descriptors above in line with the general marking guidance (page 3).

Performance in AO1 and AO2 is interdependent. An answer displaying **no** qualities of AO2 cannot be awarded more than the top of Level 1, no matter how strong performance is in AO1; markers should note that the expectation for AO1 is that candidates demonstrate both knowledge *and* understanding.

The middle mark in each level may be achieved by stronger performance in either AO1 or AO2.

Indicative content guidance

Answers must be credited according to candidates' deployment of material in relation to the qualities outlined in the mark scheme. While specific references are made in the indicative content below, this does not imply that these must be included; other relevant material must also be credited.

Relevant points may include:

- People could avoid the pain of an operation because the development of anaesthetics meant that they were, therefore, more ready to contemplate surgery.
- The development of anaesthetics meant that longer and more complex operations could be undertaken, offering hope that surgery could move beyond basic procedures such as amputations.
- The reduced risk of infection and complications in surgery using high-tech surgery, for example keyhole surgery, made surgeons and patients more positive about surgery.
- High-tech surgery made complex operations such as transplants possible, leading people to see surgery as a life-saving procedure.
- Antiseptic and aseptic techniques began to be used in the late nineteenth century and improved the survival rate after operations, changing people's attitudes towards surgery.
- The discovery of blood groups and the development of transfusions improved survival rates, meaning that surgery was more commonly accepted.

Question		
5		'In the years c1250-c1500, the physician was the most important person providing care and treatment.' How far do you agree? Explain your answer. You **may** use the following in your answer. • medical training • herbal remedies You **must** also use information of your own. **Target**: Analysis and evaluation of second order concepts: significance; change and continuity [AO2]; Knowledge and understanding of features and characteristics [AO1]. **AO2**: 10 marks **AO1**: 6 marks.
Level	**Mark**	**Descriptor**
	0	No rewardable material.
1	**1–4**	• A simple or generalised answer is given, lacking development and organisation. [AO2] • Limited knowledge and understanding of the topic is shown. [AO1] • The overall judgement is missing or asserted. [AO2]
2	**5–8**	• An explanation is given showing limited analysis and with implicit or unsustained links to the conceptual focus of the question. It shows some development and organisation of material, but a line of reasoning is not sustained. [AO2] • Accurate and relevant information is included, showing some knowledge and understanding of the period. [AO1] • The overall judgement is given but its justification is asserted or insecure. [AO2] *Maximum 7 marks for Level 2 answers that do not go beyond aspects prompted by the stimulus points.*
3	**9–12**	• An explanation is given, showing some analysis, which is mainly directed at the conceptual focus of the question. It shows a line of reasoning that is generally sustained, although some passages may lack coherence and organisation. [AO2] • Accurate and relevant information is included, showing good knowledge and understanding of the required features or characteristics of the period studied. [AO1] • The overall judgement is given with some justification, but some criteria selected for the required judgement are left implicit or not validly applied. [AO2] *Maximum 11 marks for Level 3 answers that do not go beyond aspects prompted by the stimulus points.*
4	**13–16**	• An analytical explanation is given which is directed consistently at the conceptual focus of the question, showing a line of reasoning that is coherent, sustained and logically structured. [AO2] • Accurate and relevant information is precisely selected to address the question directly, showing wide-ranging knowledge and understanding of the required features or characteristics of the period studied. [AO1] • Criteria for the required judgement are justified and applied in the process of reaching the overall judgement. [AO2] *No access to Level 4 for answers that do not go beyond aspects prompted by the stimulus points.*

Marking instructions

Markers must apply the descriptors above in line with the general marking guidance (page 3).

Performance in AO1 and AO2 is interdependent. An answer displaying **no** qualities of AO2 cannot be awarded more than the top of Level 1, no matter how strong performance is in AO1; markers should note that the expectation for AO1 is that candidates demonstrate both knowledge *and* understanding.

The first two bullet points *[AO1 and AO2]* account for 3 of the 4 marks in the level and are equally weighted; the third bullet point *[AO2]* accounts for the remaining mark. Once the level has been found, there are two steps to follow to determine the mark within the level:

- Markers should consider bullet points 1 and 2 together. Strong performance (for the level) in both would be awarded all 3 marks, while 2 marks may be achieved by stronger performance in either bullet point; weak performance would be awarded 1 mark.
- The fourth mark in each level is allocated to the bullet point 3 and should be considered independently of the award of the other marks.

Indicative content guidance

Answers must be credited according to candidates' deployment of material in relation to the qualities outlined in the mark scheme. While specific references are made in the indicative content below, this does not imply that these must be included; other relevant material must also be credited. The grouping of points below does not imply that this is how candidates are expected to structure their answers.

Relevant points that support the statement may include:

- There was a growing emphasis on the role of the physician, who had completed medical training and was, therefore, regarded as having expert knowledge.
- During this period, medical courses were developed and texts were produced to ensure physicians were properly trained and knowledgeable.
- The physician would prescribe the medicine to be mixed by the apothecary, or decide if the patient needed to be bled by the barber surgeon, so the physician directed the treatment that was given.

Relevant points to counter the statement may include:

- Medical training remained based on Galen's ideas and therefore treatment by the physician was often not effective.
- Throughout the period, care and treatment for most people was provided by the women in the family, using herbal remedies or folk remedies.
- People had to pay for consulting the physician and then pay for the medicine so treatment from the physician remained inaccessible for most ordinary people.
- The apothecary would often sell ready-mixed medicine to people who could not afford to consult a physician, meaning that for most people, he was a more important individual than the physician.
- Monks and nuns ran increasing numbers of hospitals that provided care and some treatment for the sick.

Question		
6		'In the years c1800-present, the work of Pasteur was the most significant development in understanding the causes of illness.' How far do you agree? Explain your answer. You **may** use the following in your answer: • Germ Theory • DNA You **must** also use information of your own. **Target**: Analysis and evaluation of second order concepts: change; significance [AO2]; Knowledge and understanding of features and characteristics [AO1]. **AO2**: 10 marks. **AO1**: 6 marks.
Level	**Mark**	**Descriptor**
	0	No rewardable material.
1	**1–4**	• A simple or generalised answer is given, lacking development and organisation. [AO2] • Limited knowledge and understanding of the topic is shown. [AO1] • The overall judgement is missing or asserted. [AO2]
2	**5–8**	• An explanation is given showing limited analysis and with implicit or unsustained links to the conceptual focus of the question. It shows some development and organisation of material, but a line of reasoning is not sustained. [AO2] • Accurate and relevant information is included, showing some knowledge and understanding of the period. [AO1] • The overall judgement is given but its justification is asserted or insecure. [AO2] *Maximum 7 marks for Level 2 answers that do not go beyond aspects prompted by the stimulus points.*
3	**9–12**	• An explanation is given, showing some analysis, which is mainly directed at the conceptual focus of the question. It shows a line of reasoning that is generally sustained, although some passages may lack coherence and organisation. [AO2] • Accurate and relevant information is included, showing good knowledge and understanding of the required features or characteristics of the period studied. [AO1] • The overall judgement is given with some justification, but some criteria selected for the required judgement are left implicit or not validly applied. [AO2] *Maximum 11 marks for Level 3 answers that do not go beyond aspects prompted by the stimulus points.*
4	**13–16**	• An analytical explanation is given which is directed consistently at the conceptual focus of the question, showing a line of reasoning that is coherent, sustained and logically structured. [AO2] • Accurate and relevant information is precisely selected to address the question directly, showing wide-ranging knowledge and understanding of the required features or characteristics of the period studied. [AO1] • Criteria for the required judgement are justified and applied in the process of reaching the overall judgement. [AO2] *No access to Level 4 for answers that do not go beyond aspects prompted by the stimulus points.*

Marking instructions

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Relevant points that support the statement may include:

- Pasteur proved that illness was not caused by spontaneous generation or by miasma, overturning existing accepted ideas; this was a turning point in understanding the causes of illness.
- Pasteur's Germ Theory laid a foundation which allowed other researchers such as Koch to identify the specific microbes causing various diseases.
- Pasteur's work formed the basis of an explanation of how cholera was spread by polluted water, making sense of Snow's work on the Broad Street pump, which had not been fully understood before.
- Pasteur's work could be used to understand the link between living conditions, hygiene and health, which was an extremely significant development in understanding the causes of illness.

Relevant points to counter the statement may include:

- An understanding of the structure of DNA has led to the realisation that not all illnesses are caused by microbes and some are the result of genetic conditions.
- The Human Genome Project has led to the identification of specific faulty genes causing illnesses, such as breast cancer.
- An earlier breakthrough occurred when Snow's investigation into cholera and the Broad Street pump led to an understanding that disease could be spread by contaminated water; this was an important development even though there was not a scientific explanation at that point.
- Scientific research has shown that some illnesses are linked to lifestyle, for example, smoking and lung cancer, alcohol and liver disease, obesity and heart disease.

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